# Meridian Shift Fatigue Review 2025

**Meridian Health Trust · Workforce Wellbeing Unit** Report to the Trust Board · November 2025 · Fictional document for the Horizon demo world "Meridian Council"

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## Summary

Fatigue is no longer an occasional complaint among the Trust's clinical staff. It is a measurable, rising condition with a direct line to patient safety, sickness absence and resignations. This review followed 1,140 clinical staff for ten months and found that the length of consecutive shift blocks, not total hours alone, is the strongest predictor of how tired staff say they are.

The headline findings:

- Staff working more than three consecutive long shifts reported fatigue scores 38% higher than colleagues on shorter blocks.
- High burnout in the emergency department rose from 29% of staff in January to 41% in October.
- Staff who slept under six hours before a shift were twice as likely to report a near-miss medication error that week.
- Two wards that piloted a compressed rota saw burnout fall by roughly a fifth in twelve weeks, with no loss of patient throughput.
- Voluntary nursing resignations reached 14% over the year, and each replacement costs about four months of salary.

The review recommends extending the compressed-rota pilot to three further departments, protecting rest days from short-notice cancellation, and reporting fatigue scores to the Board every quarter.

## 1. Scope and method

This review covers 1,140 clinical staff across four Meridian Health Trust sites between January and October 2025. Fatigue was measured with a fortnightly self-report scale and matched against rota data. All names and units are anonymised.

The four sites are Meridian General (the acute hospital and the Trust's only emergency department), Eastbank Community Hospital, the Riverside Rehabilitation Unit and the Northgate Day Surgery Centre. Participation was voluntary; 71% of eligible clinical staff took part, and response rates held above 60% in every fortnight.

The self-report scale has nine items: sleepiness at the start and end of a shift, difficulty concentrating, irritability, physical exhaustion, headaches, appetite, sleep quality and a single "how tired are you right now" question. Each item is scored 0 to 4, so totals run from 0 to 36. Burnout was measured separately, once a quarter, with a validated twelve-item inventory. The review's "high burnout" threshold is the inventory's published cut-off.

Rota data came from the Trust's e-rostering system. For every respondent we recorded shift length, the number of consecutive shifts, the hours of rest between shifts, cancelled rest days and any short-notice changes (less than 48 hours).

### 1.1 What this review does not measure

The review relies on what staff report. It does not measure fatigue objectively, and it cannot prove that long blocks cause errors. It shows associations that are consistent across sites, departments and months. Where we discuss costs, we use the Trust finance office's standard figures.

## 2. Shift patterns and fatigue

The single clearest pattern in the data is the effect of long blocks.

Staff working more than three consecutive long shifts reported fatigue scores 38% higher than colleagues on shorter blocks. The effect was strongest in emergency and acute medicine, where rest days were most often cancelled at short notice.

A "long shift" in this review is any shift of eleven hours or more. Across the Trust, 46% of clinical shifts were long shifts. In the emergency department the figure was 78%.

Total weekly hours mattered less than the way those hours were arranged. Two nurses working the same 37.5 hours could report very different fatigue: the one who worked four long shifts in a row, then had three days off, reported less fatigue than the one whose long shifts were scattered across the week with single rest days in between. Single rest days, especially after a night shift, were rarely restful.

### 2.1 Short-notice changes

Rest days cancelled with less than 48 hours' notice were the second strongest predictor of fatigue. In emergency and acute medicine, one rest day in nine was cancelled at short notice, usually to cover sickness. In community and day-surgery settings the figure was one in forty.

## 3. Burnout

Self-reported burnout rose in every quarter of the review period. By October, 41% of emergency department staff met the review's threshold for high burnout, up from 29% in January.

Across the whole Trust, high burnout rose from 22% to 30%. The rise was steepest among staff in their first five years after qualifying and among those with caring responsibilities at home.

Burnout and fatigue moved together but were not the same thing. Some staff with high fatigue scores did not report burnout, usually those on predictable rotas who described their tiredness as "physical, not mental". Burnout was most common where fatigue combined with unpredictability: the sense that rest could be taken away at any time.

### 3.1 What staff said

In free-text answers, the words that appeared most often next to "burnout" were "cancelled", "short-staffed", "no break" and "guilt". Many respondents said they felt guilty taking sick leave because they knew a colleague would lose a rest day to cover them.

## 4. Sleep and patient safety

Poor sleep was the most common complaint, named by 63% of respondents. Staff who slept under six hours before a shift were twice as likely to report a near-miss medication error that week.

Night shifts were the main cause of short sleep. Staff moving from nights back to days with only one rest day in between averaged under five hours of sleep on the first night home.

A near miss is an error caught before it reaches the patient: the wrong dose drawn up and noticed, the wrong patient's chart opened and closed again. The Trust's incident system does not link incidents to rotas, so the review used a short anonymous weekly question instead. The association held in every department and at every site.

### 4.1 The commute home

Twelve percent of respondents said they had nodded off while driving home after a night shift at least once during the review period. Three reported minor collisions. The Trust's car park sleep pods, piloted at Meridian General since March, were used by fewer than one in twenty eligible staff, mostly because staff wanted to get home to children.

## 5. The cost of fatigue

Fatigue-related problems among staff themselves, from headaches and gastric complaints to minor injuries on the commute home, cost the Trust an estimated 2,300 lost shifts.

That estimate counts sickness absence recorded with a fatigue-related reason, plus the share of other short absences that staff attributed to tiredness in the survey. It does not include presenteeism, staff who came in tired and worked below their usual pace, which the review could not measure.

At the Trust's standard cost of covering a lost shift with bank or agency staff, 2,300 shifts is roughly the annual cost of forty full-time nurses.

## 6. The compressed-rota pilot

Two wards piloted a compressed rota: four longer days, then three days off. After twelve weeks, burnout scores on both wards fell by roughly a fifth, sleep quality improved, and patient throughput stayed within normal variation.

The pilot wards were Ward 7 (acute medicine) and the emergency department's observation unit. Staff chose to join. The rota guaranteed the three rest days: they could not be cancelled except in a declared major incident.

What changed after twelve weeks:

- Burnout scores fell by 19% on Ward 7 and 22% on the observation unit.
- Average sleep before a shift rose by 40 minutes.
- Short-notice cancellations of rest days fell to zero, by design.
- Patient throughput, length of stay and readmissions stayed within normal variation for those wards.

Staff on the pilot wards described the guaranteed rest days, not the longer shifts, as the change that mattered most.

## 7. Transition costs

The pilot wards needed 6% more agency hours during the transition month to cover handovers. That cost fell back to baseline by week eight, once teams had settled the new handover pattern.

Most of the transition cost came from overlapping handovers while two rota patterns ran side by side, and from a period of retraining on the new handover checklist. Ward managers recommended a two-week planning period before any further rollout, and a single start date per ward rather than a phased switch.

The pilot did not test round-the-clock coverage on a whole-department scale. The emergency department proper, as opposed to its observation unit, would need further modelling before a compressed rota could be offered there.

## 8. Retention

Voluntary resignations among nursing staff reached 14% over the year. The Trust's finance office puts each replacement at roughly four months of salary once recruitment, induction and supervised practice are counted.

The rate was highest in the emergency department (19%) and lowest in day surgery (6%). Two thirds of leavers had been with the Trust for under five years.

Replacing a nurse is not only a cost. New starters need supervision, which takes experienced staff away from patients during the first months. Wards with high turnover were also the wards with the most short-notice rest-day cancellations, a cycle the review describes as "exhaustion feeding exhaustion".

## 9. Why staff leave

Exit interviews named workload and unpredictable rotas as the two leading reasons for leaving, ahead of pay. Several leavers said they would have stayed for a guaranteed rest pattern.

Of 162 exit interviews analysed, 58% named workload, 51% named rota unpredictability, 37% named pay and 22% named a lack of career development. Respondents could give more than one reason.

Leavers who moved to other hospitals often cited a "fixed pattern" or "self-rostering" as the reason for choosing their new employer.

## 10. Recommendations

The review recommends extending the compressed-rota pilot to three further departments, protecting rest days from short-notice cancellation, and reporting fatigue scores to the Trust board every quarter.

In detail:

1. **Extend the pilot** to acute surgery, the Eastbank inpatient wards and the Riverside Rehabilitation Unit, with a two-week planning period and a single start date for each.
2. **Protect rest days.** Short-notice cancellation should need a manager's sign-off and be recorded, and no member of staff should lose more than one rest day a quarter.
3. **Limit long blocks** to three consecutive long shifts, except by the employee's own request.
4. **Two rest days after nights** before any day shift.
5. **Report quarterly** to the Board: fatigue scores, burnout prevalence, rest-day cancellations and resignations, by department.
6. **Model the emergency department** separately before offering it a compressed rota.

## Appendix: the fatigue scale

Nine items, each scored 0 (not at all) to 4 (extremely): sleepiness at the start of the shift; sleepiness at the end; difficulty concentrating; irritability; physical exhaustion; headache; change in appetite; poor sleep quality; and "how tired are you right now". Scores of 18 and above are treated as high fatigue in this review.